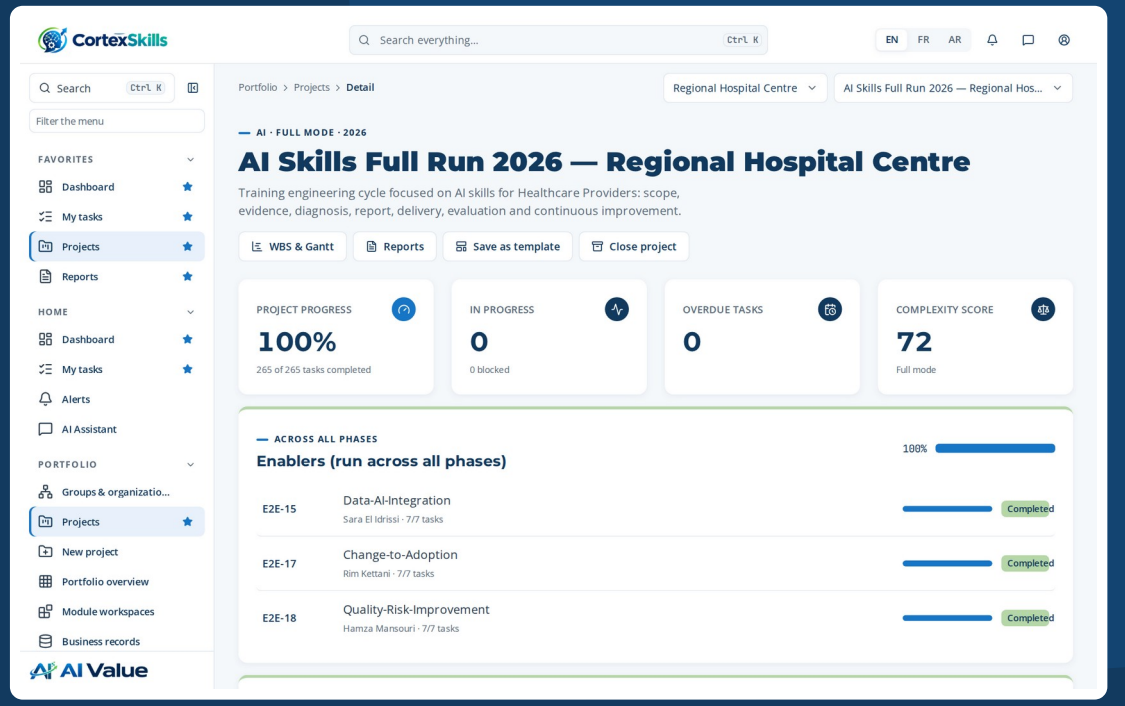
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FULL CASE · HEALTHCARE · 2026

AI skills training engineering at a public hospital

Regional Hospital Centre: the project, the Training Engineering Report, the Training Plan and every document produced with CortexSkills



MEASURABLE · GOVERNED · SUSTAINABLE

AGENDA

How the case is presented

01

The hospital and the project

Context, charter, scope, method, schedule

02

Diagnosis

Questionnaires, maturity, gaps, needs

03

Training Engineering Report

Structure, findings, roadmap, risks

04

Training Plan

Programs, agendas, design, budget

05

Delivery, evaluation and audit

Sessions, Kirkpatrick, ROI, audit

06

Documents produced

37 published documents, traced

EXECUTIVE SUMMARY

A complete AI skills cycle, documented end to end

Regional Hospital Centre · 3,400 employees · AI Skills Full Run 2026 — Regional Hospital Centre

100 %
of the run completed — 35 end-to-end processes

37 / 40
people answered the questionnaires (93 %)

6
trainings in 2 programs, prioritized by impact

1,260,000
training budget; 1,192,662 spent

What the case shows

- Diagnosis: “Information systems” is the weakest axis (2 / 5); the largest skill gap is “AI bed and staff capacity planning” (average gap 3.0).
- 10 training needs were validated and grouped into 6 themes, deployed in 4 roadmap initiatives over three waves.
- Kirkpatrick results 86 % · 93 % · 84 %; training ROI 108 % (target ≥ 100%).
- 37 documents published, 1,060 pages; the Training Engineering Report alone has 198 pages.

01

The hospital and the project

Context, charter, scope, method and schedule

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CONTEXT

Regional Hospital Centre

Regional Hospital Centre operates in the Healthcare Providers sector, in Région, with 3400 employees.

Topic	Content
Vision	Become the reference of Healthcare Providers in the region, with teams trained to the best standards.
Mission	Deliver safe, compliant service through skilled people and controlled processes.
Values	Commitment · Excellence · Sharing
Context	Strong competition, stricter ISO 7101 audits and the arrival of AI triage and clinical decision support.
Products — existing	Core services of Clinical Care & Nursing
Products — new	Digital services supported by AI triage and clinical decision support
Market segments	Large accounts · Public sector · Individual customers

Organization and headcount

Function	People
Operations	3
Quality & HSE	3
Finance	2
Human Resources	2
Sales & Marketing	2
Information Systems	2
Clinical Care & Nursing	2

A public hospital under pressure: why skills come first

What the functional SWOTs and the strategic objectives say

External pressure

- Stricter ISO 7101 audits
- Departure of key experts

Opportunities to seize

- Demand for digital services
- Training contracts refunded by OFPPT
- AI bed and staff capacity planning

Weak points today

- Low use of AI triage and clinical decision support
- Reporting kept in spreadsheets
- Little versatility between roles
- Low use of AI bed and staff capacity planning
- Low use of AI medical imaging assistance

What the direction commits to

- SO-1: Adopt AI triage and clinical decision support across Clinical Care & Nursing by end 2027
- SO-2: Zero major findings in ISO 7101 audits

Source: SWOT of TER 1.4, functional SWOT records (E2E-25) and strategic objectives of the Scope of Work.

Key dates and SWOT of the hospital

2008

Company founded

2014

Second site opened

2019

ISO 7101 certification

2025

Digital transformation program launched

Strengths

- Experienced, committed team
- Good knowledge of the market
- Stable processes

Areas for improvement

- Low use of AI triage and clinical decision support
- Reporting kept in spreadsheets
- Little versatility between roles
- Low use of AI bed and staff capacity planning
- Low use of AI medical imaging assistance

Opportunities

- Demand for digital services
- Training contracts refunded by OFPPT

Threats

- Stricter ISO 7101 audits
- Departure of key experts

Source: TER sections 1.3 and 1.4.

AI Skills Full Run 2026 — Regional Hospital Centre

Training engineering cycle focused on AI skills for Healthcare Providers: scope, evidence, diagnosis, report, delivery, evaluation and continuous improvement.

72
complexity score (out of 100): full run with gates

40
stakeholders registered and consulted

17
scope criteria approved in the Scope of Work

MS · MO · OP
decision levels covered (strategic, middle, operational)

Strategic objectives

- SO-1 — Adopt AI triage and clinical decision support across Clinical Care & Nursing by end 2027 (horizon 2027-12-31)
- SO-2 — Zero major findings in ISO 7101 audits (horizon 2026-12-31)

Functions and decision levels in scope

Focus: AI · Decision levels: MS, MO, OP · Approval: Approved (2026-04-30, Soukaina Ouazzani)

Function	MS — strategic	MO — middle	OP — operational
Operations	●	●	●
Quality & HSE	●	●	—
Finance	●	—	●
Human Resources	—	●	●
Sales & Marketing	●	●	●
Information Systems	●	●	—
Clinical Care & Nursing	●	—	●

How the scope is set

Each function × decision-level cell is a weighted criterion of the Scope of Work. 16 criteria were approved by the sponsor before any questionnaire was sent; every later table of the report is filtered by this scope.

Source: TER section 1.8.

METHOD

Six phases, each closed by a gate with its deliverables

The training engineering mission follows the phases below, each closed by a gate with its deliverables.

Phase	End-to-end processes	Progress
1. Scope & evidence	E2E-32 · E2E-33 · E2E-19 · E2E-24	100%
2. Diagnosis	E2E-20 · E2E-25 · E2E-26 · E2E-27 · E2E-28 · E2E-29 · E2E-02 · E2E-21	100%
3. Training Engineering Report & plan	E2E-01 · E2E-03 · E2E-30 · E2E-22 · E2E-23 · E2E-16 · HCPR-E2E-02	100%
4. Design & delivery	E2E-04 · E2E-05 · E2E-06 · E2E-13 · E2E-14	100%
5. Evaluation, compliance & impact	E2E-07 · E2E-08 · E2E-09 · HCPR-E2E-01	100%
6. Talent, knowledge & continuous improvement	E2E-10 · E2E-11 · E2E-12 · E2E-31	100%

Source: Training Engineering Report, section 1.5 "The current mission".

THE RUN IN FIGURES

35 end-to-end processes, 265 tasks, all completed

Each process has an owner, its tasks, its dates and the document it produces

Phase	Processes	Tasks done	Start	End	Owners
Enablers (run across all phases)	3	21 / 21	2026-04-10	2026-05-01	3
Scope & evidence	4	37 / 37	2026-04-10	2026-05-16	4
Diagnosis	8	62 / 62	2026-05-08	2026-06-07	3
Training Engineering Report & plan	7	50 / 50	2026-06-05	2026-07-05	3
Design & delivery	5	39 / 39	2026-07-03	2026-07-30	3
Evaluation, compliance & impact	4	26 / 26	2026-07-31	2026-08-24	2
Talent, knowledge & continuous improvement	4	30 / 30	2026-08-28	2026-09-21	4



265 / 265

tasks completed, none overdue or blocked



14

process owners from the project team



37

published documents, one per process and more

Source: project workspace (phases, processes, tasks).

PROCESSES AND OWNERS

Who carried each process of the run

E2E	Process — document produced	Phase	Owner	Tasks
E2E-15	Data, AI and integration governance	Enablers (run across all phas…	Sara El Idrissi	7/7
E2E-17	Change and adoption plan	Enablers (run across all phas…	Rim Kettani	7/7
E2E-18	Quality, risk and improvement report	Enablers (run across all phas…	Hamza Mansouri	7/7
E2E-32	Scope of work and stakeholder register	Scope & evidence	Leila Chraibi	9/9
E2E-33	Questionnaire campaign report	Scope & evidence	Asmae Fassi	12/12
E2E-19	Consulting engagement closure report	Scope & evidence	Hamza Benjelloun	9/9
E2E-24	Competitive intelligence note	Scope & evidence	Omar Benjelloun	7/7
E2E-20	Multi-axis diagnostic and maturity report	Diagnosis	Hamza Benjelloun	10/10
E2E-25	Job architecture and competency map	Diagnosis	Soukaina Ouazzani	8/8
E2E-26	Forward planning of jobs and skills (GPEC)	Diagnosis	Soukaina Ouazzani	8/8
E2E-27	Training demand consolidation	Diagnosis	Leila Chraibi	7/7
E2E-28	Performance assessment and RAG dashboard	Diagnosis	Leila Chraibi	7/7
E2E-29	Functional SWOT and improvement axes	Diagnosis	Leila Chraibi	7/7
E2E-02	Skills gap diagnosis	Diagnosis	Leila Chraibi	7/7
E2E-21	Previous plan impact evaluation	Diagnosis	Leila Chraibi	8/8
E2E-01	Annual L&D plan and budget	Training Engineering Report &…	Leila Chraibi	8/8
E2E-03	Training needs report and roadmap	Training Engineering Report &…	Leila Chraibi	10/10

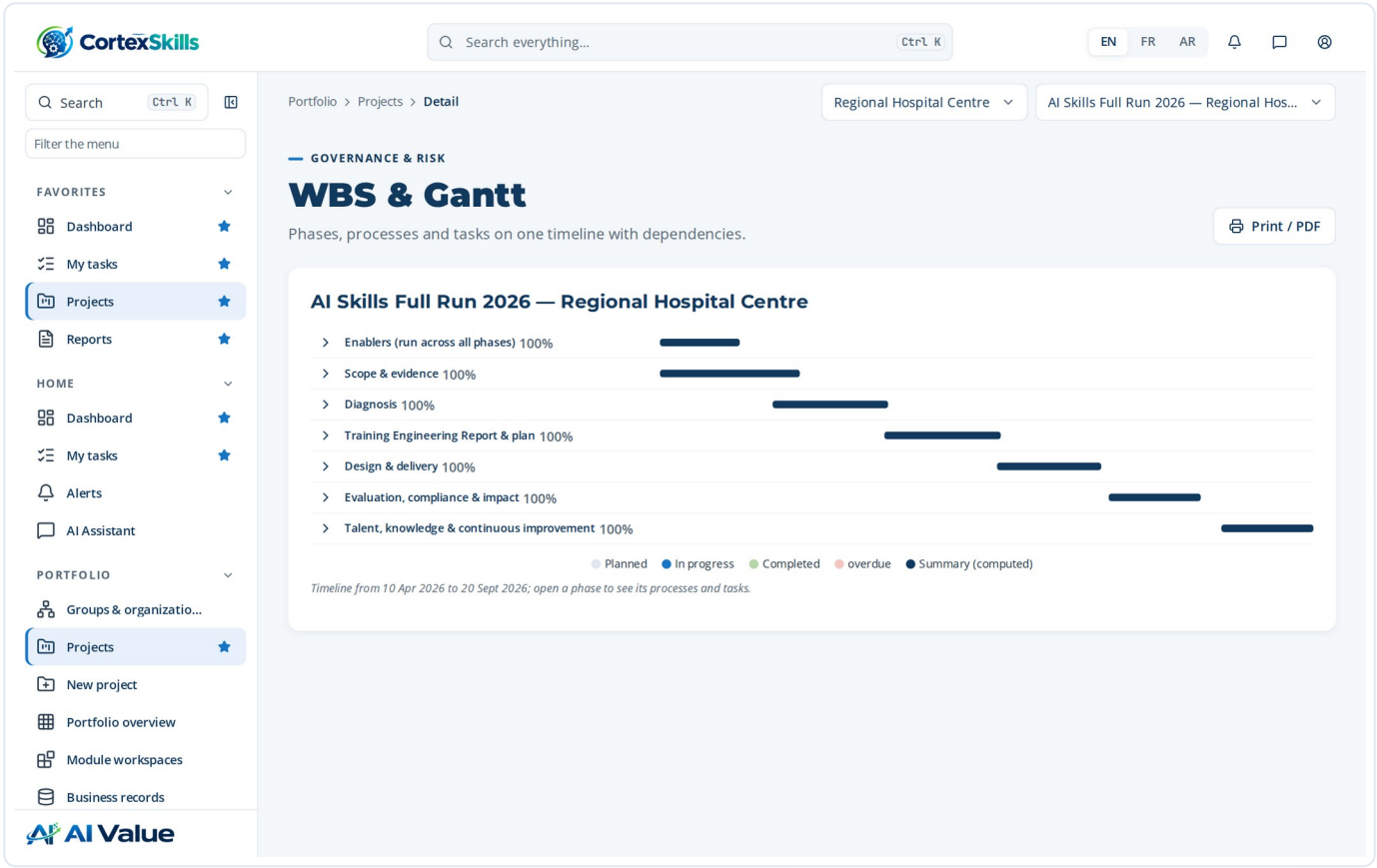
The processes of the run (continued)

E2E	Process — document produced	Phase	Owner	Tasks
E2E-22	Multi-site deployment plan	Training Engineering Report &…	Leila Chraibi	8/8
E2E-23	Regulatory submission file	Training Engineering Report &…	Kawtar Rami	7/7
E2E-16	Vendor management file	Training Engineering Report &…	Imane Naciri	6/6
HCPR-E2E-02	Healthcare-specific process (feeds the TER and the audit)	Training Engineering Report &…	Leila Chraibi	4/4
E2E-04	Curriculum design document	Design & delivery	Leila Chraibi	8/8
E2E-05	Training delivery report	Design & delivery	Leila Chraibi	9/9
E2E-06	Learner engagement report	Design & delivery	Leila Chraibi	8/8
E2E-13	Content catalogue and lifecycle	Design & delivery	Rim Mansouri	7/7
E2E-14	Extended enterprise training report	Design & delivery	Hajar Chraibi	7/7
E2E-07	Training evaluation report	Evaluation, compliance & impa…	Leila Chraibi	8/8
E2E-08	ROI and business impact report	Evaluation, compliance & impa…	Leila Chraibi	7/7
E2E-09	Compliance and certification register	Evaluation, compliance & impa…	Kawtar Rami	7/7
HCPR-E2E-01	Healthcare-specific process (feeds the TER and the audit)	Evaluation, compliance & impa…	Leila Chraibi	4/4
E2E-10	Internal mobility and career paths	Talent, knowledge & continuou…	Soukaina Ouazzani	8/8
E2E-11	Onboarding programme record	Talent, knowledge & continuou…	Hamza Lahlou	7/7
E2E-12	Knowledge capture and transfer record	Talent, knowledge & continuou…	Omar Alaoui	7/7

Work packages and the Gantt chart

Work package	Start	End	Progress
Scope & evidence	2026-04-10	2026-05-07	100 %
Diagnosis	2026-05-08	2026-06-04	100 %
Report & plan	2026-06-05	2026-07-02	100 %

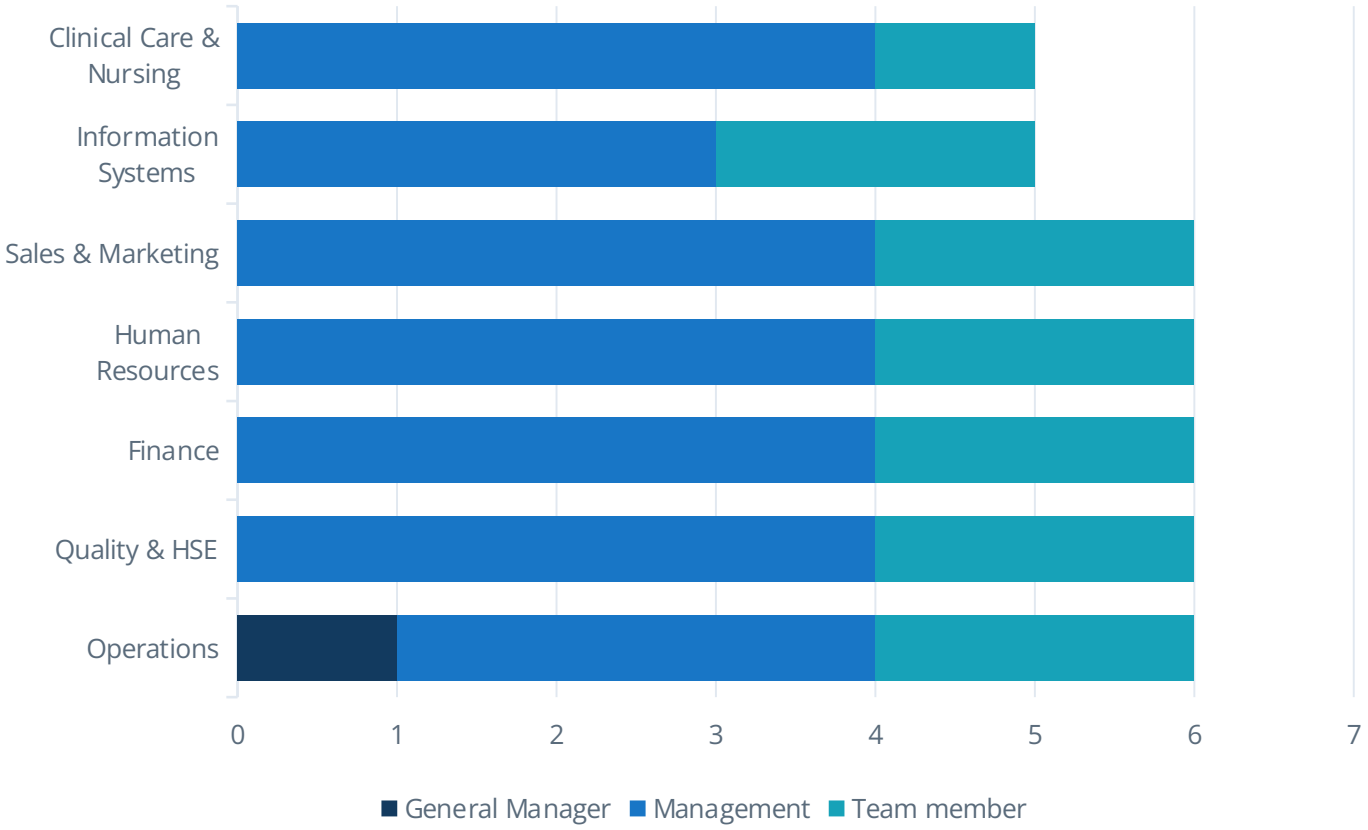
Sessions	Date
S-01 AI triage and clinical decision …	2026-07-19
S-02 AI medical imaging assistance	2026-07-26
S-03 AI triage and clinical decision …	2026-08-02
S-04 AI medical imaging assistance	2026-08-09



WBS and Gantt of the project in CortexSkills.

40 people consulted across every function

40 people were consulted: 1 from general management, 26 managers and 13 staff members.



People consulted by function and population (TER 1.7).

Channel	People
Face-to-face	1
Combination	26
Email	13

Consent recorded for 40 of 40 people before any answer was stored.

02

Diagnosis

Who answered, where the hospital stands, which skills are missing

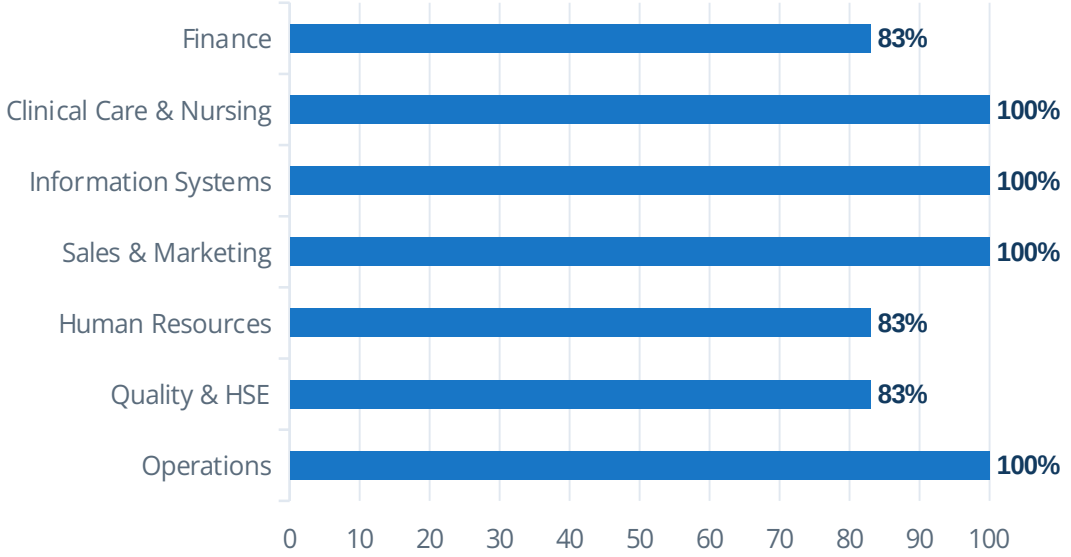
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QUESTIONNAIRE CAMPAIGN

37 of 40 people answered

40 people were invited to answer; 37 answered.

Function	General Manager	Management	Team member	Responded
Operations	1	3	2	6
Quality & HSE	0	4	2	5
Human Resources	0	4	2	5
Sales & Marketing	0	4	2	6
Information Systems	0	3	2	5
Clinical Care & Nursing	0	4	1	5
Finance	0	4	2	5



Response rate by function.

Channel	Invited	Responded	Rate	Completeness
Face-to-face	12	12	100 %	95 %
Application	23	23	100 %	91 %
Email	2	2	100 %	77 %

Source: TER sections 1.6, 1.12 and 1.13.

What the teams said about the last training plan

What did you notice about the n-1 training plan?

• Trainings were chosen late in the year and with little link to our objectives.
• Good participation, but little follow-up after the sessions.

What were the strengths of the n-1 plan?

• Sessions held on site, close to the teams.
• Practical trainers who knew our sector.

What improvements did the n-1 plan bring?

• Supervisors use the new procedures.
• Fewer errors on ISO 7101 records.

What were the challenges of the n-1 plan?

• Measuring the effect of the trainings.
• Releasing people from the job during peaks.

What did you appreciate and like about the n-1 plan?

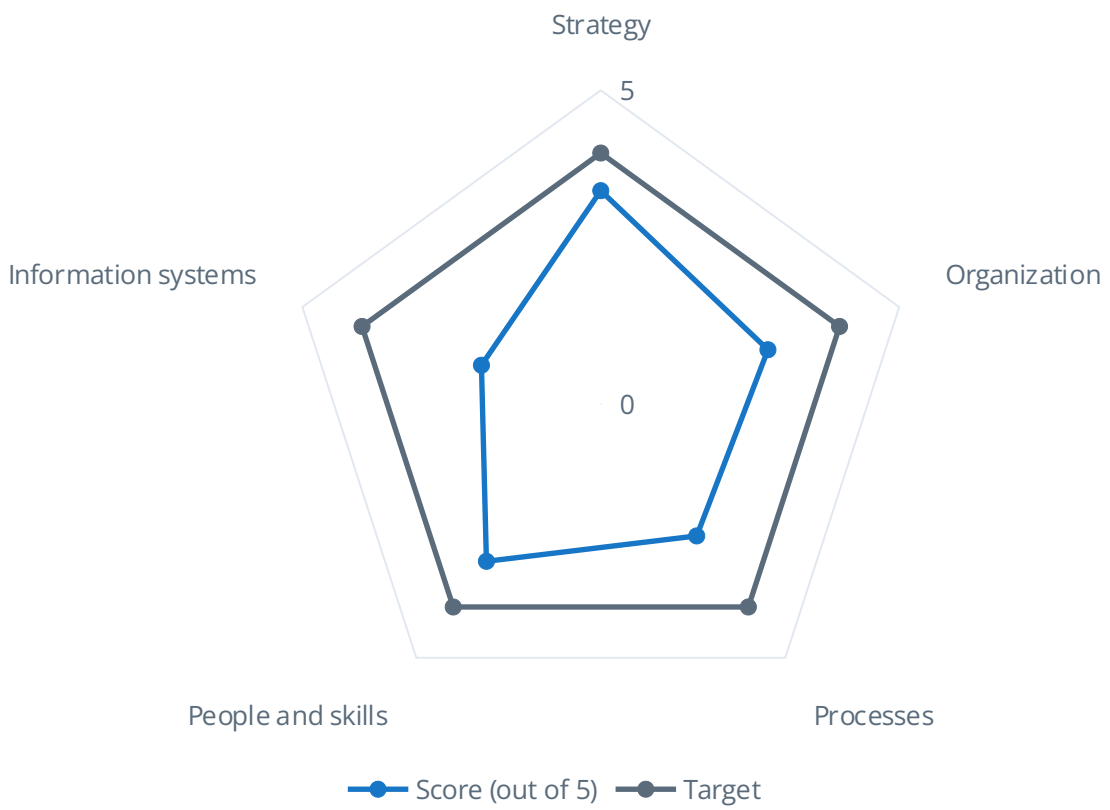
• The workshops on real cases.
• The exchange between sites.

For a new training plan, what would you like to see done differently?

• Shorter half-day formats with a workshop each time.
• A plan built from the needs of each function.

Source: TER chapter 2 — 59 trainings attended are listed in section 2.1.

Five axes scored, maturity one level below target



Diagnostic score by axis against target level 4 (TER 1.9).

Axis	Score	Reading
Strategy	3.4	Strength to build on
Organization	2.8	Partly in place
Processes	2.6	Partly in place
People and skills	3.1	Partly in place
Information systems	2	Priority for development

Model	Current	Target	Gap
CMM maturity (CMM)	3	4	1
Leadership maturity (Leadership)	3	4	1

Maturity models (TER 1.10).

Where each function stands at each decision level

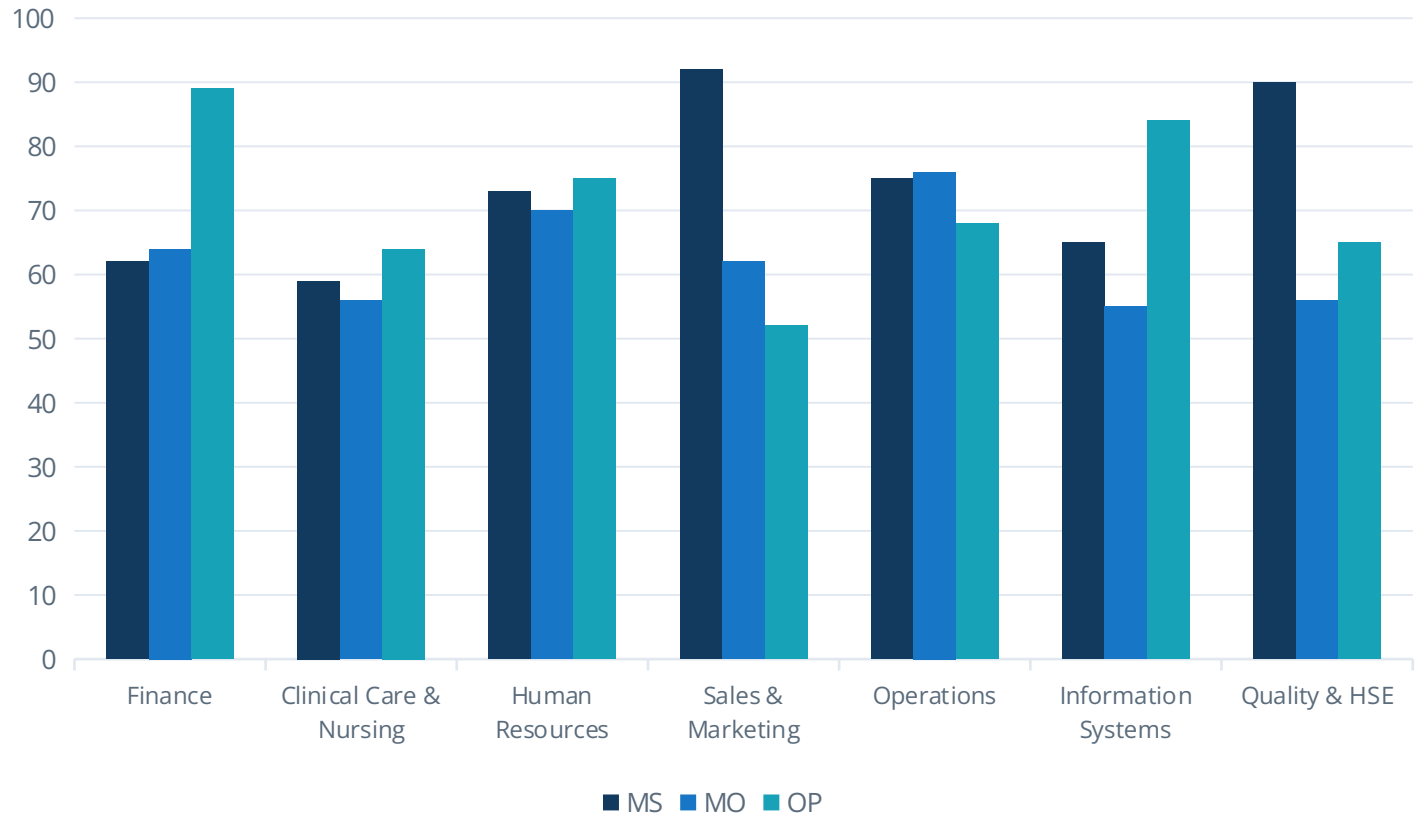
Performance by decision level: MS strategic management, MO operational management, OP operational.

Function	MS	MO	OP	Main gaps
Operations	75% Amber	76% Amber	68% Red	AI triage and clinical decision support · AI medical imaging assistance · AI bed and staff cap…
Quality & HSE	90% Green	56% Red	65% Red	AI medical imaging assistance · AI bed and staff capacity planning · AI triage and clinical de…
Finance	62% Red	64% Red	89% Amber	AI bed and staff capacity planning · AI triage and clinical decision support · AI medical imag…
Human Resources	73% Amber	70% Amber	75% Amber	AI triage and clinical decision support · AI medical imaging assistance · AI bed and staff cap…
Sales & Marketing	92% Green	62% Red	52% Red	AI medical imaging assistance · AI bed and staff capacity planning · AI triage and clinical de…
Information Systems	65% Red	55% Red	84% Amber	AI bed and staff capacity planning · AI triage and clinical decision support · AI medical imag…
Clinical Care & Nursing	59% Red	56% Red	64% Red	AI triage and clinical decision support · AI medical imaging assistance · AI bed and staff cap…

Maturity heatmap on the fixed red → green status scale (TER 3.2).

21 cells scored: function × decision level

Score in % of the expected performance, from the assessment records (E2E-26)



Performance score by function and decision level (%).

Cell	%	Gap noted
Sales & Marketing — OP	52	AI triage and clinical decision support
Information Systems — MO	55	AI triage and clinical decision support
Quality & HSE — MO	56	AI bed and staff capacity planning
Clinical Care & Nursing — MO	56	AI medical imaging assistance
Clinical Care & Nursing — MS	59	AI triage and clinical decision support
Finance — MS	62	AI bed and staff capacity planning

The six weakest cells, with the gap noted.

Strengths and weaknesses, function by function

2 SWOTs validated with the function heads

Operations — Validated

Strengths

• Experienced team
• stable processes

Weaknesses

• Low use of AI triage and clinical decision support

Opportunities

• AI bed and staff capacity planning

Threats

• Stricter ISO 7101 audits

Quality & HSE — Validated

Strengths

• Experienced team
• stable processes

Weaknesses

• Low use of AI triage and clinical decision support

Opportunities

• AI bed and staff capacity planning

Threats

• Stricter ISO 7101 audits

Source: FunctionalSwot records (E2E-25), feeding the improvement axes.

The competences people asked for, by number of mentions

Consolidated from 32 responses.



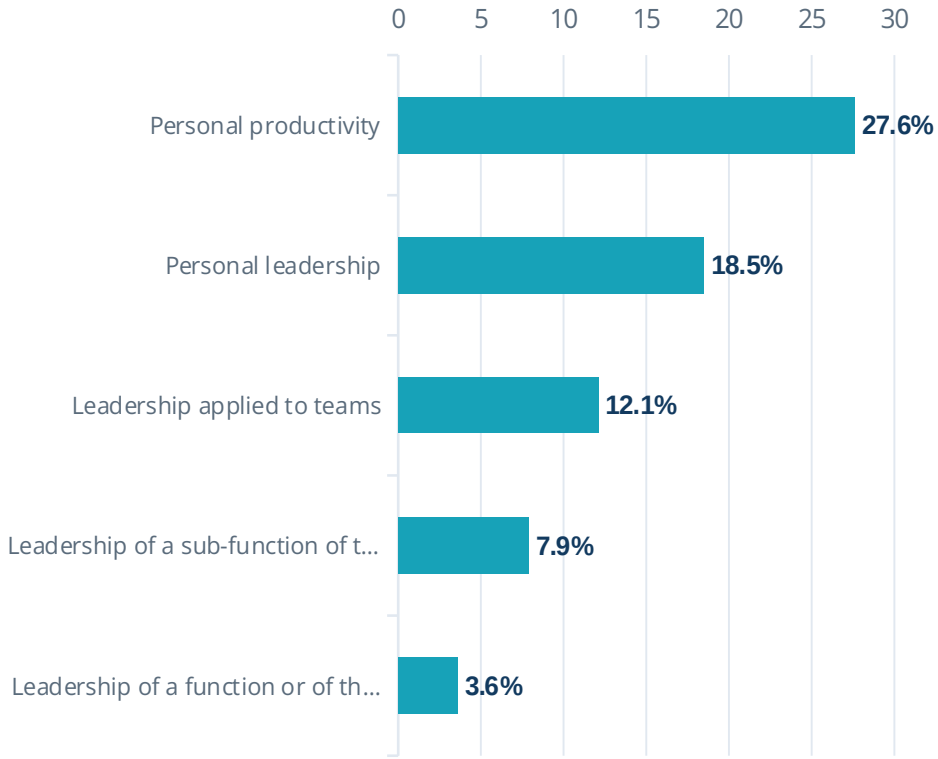
Mentions across all answers (TER 3.3).

Reading

The two AI clinical themes lead (108 and 103 mentions), followed by team communication and problem solving: the plan pairs each AI training with practical workshops on the hospital's own cases.

Self-assessment of the respondents, weakest first

Competence	Average (1–5)	n
Self-control and excellence	3.2	31
Continuous quality improvement — conflict resolution and solutions	3.2	29
Coaching others towards excellence	3.3	29
Management by values	3.3	30
Enterprise management: macro view of the company's businesses and fun…	3.3	20
Training and effectiveness	3.3	30
Self-motivation and energy	3.4	31
Communication with oneself	3.4	31

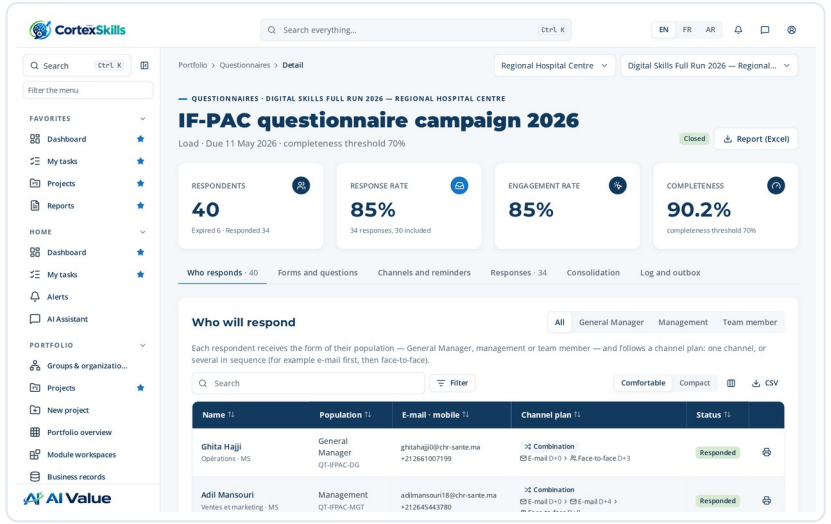


Leadership levels reached (TER 3.7).

From measured gaps to a validated needs register

Competency	People	Avg gap	Critical
AI medical imaging assistance	3	2.0	0
AI bed and staff capacity planning	2	3.0	2
AI triage and clinical decision support	3	1.0	0

Skill gaps by competency (TER 3.40).

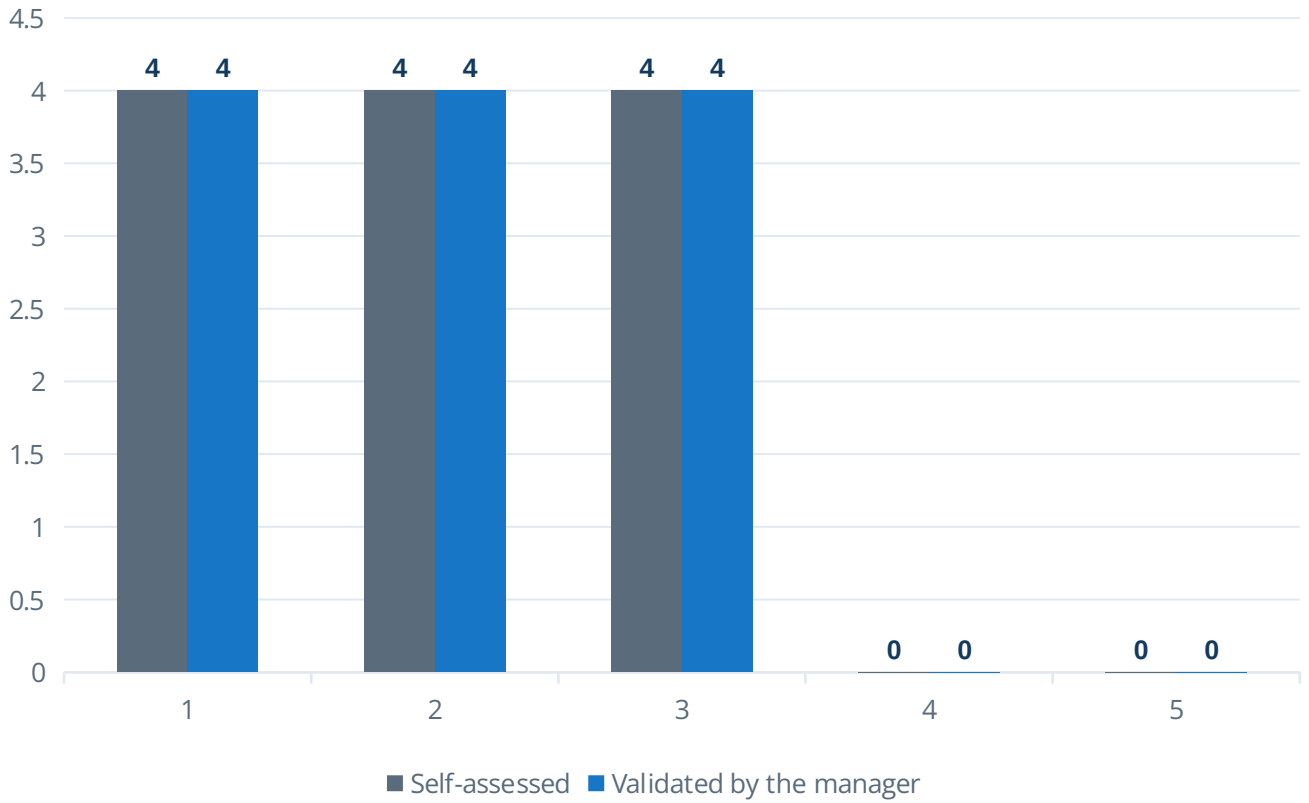


#	Theme	Source	Impact	Status
1	AI bed and staff capacity planning	Diagnostic	5	Validated
2	Responsible AI and data protection	Skills gap	5	Validated
3	AI bed and staff capacity planning	GPEC	5	Validated
4	AI medical imaging assistance	Demand	4	Validated
5	Prompting and generative AI at work	SWOT	4	Validated
6	AI medical imaging assistance	Diagnostic	4	Validated
7	AI triage and clinical decision support	Skills gap	3	Validated
8	AI literacy for all staff	GPEC	3	Validated
9	AI triage and clinical decision support	Demand	3	Validated
10	AI literacy for all staff	SWOT	3	Validated

Training needs register (TER 3.41): every need traced to its source.

Self-assessed and validated levels side by side

12 assessments of the campaign “Skills campaign 2026”, closed on 2026-05-20



Number of assessments at each level.

2.0 → 2.0

average level, self-assessed → validated

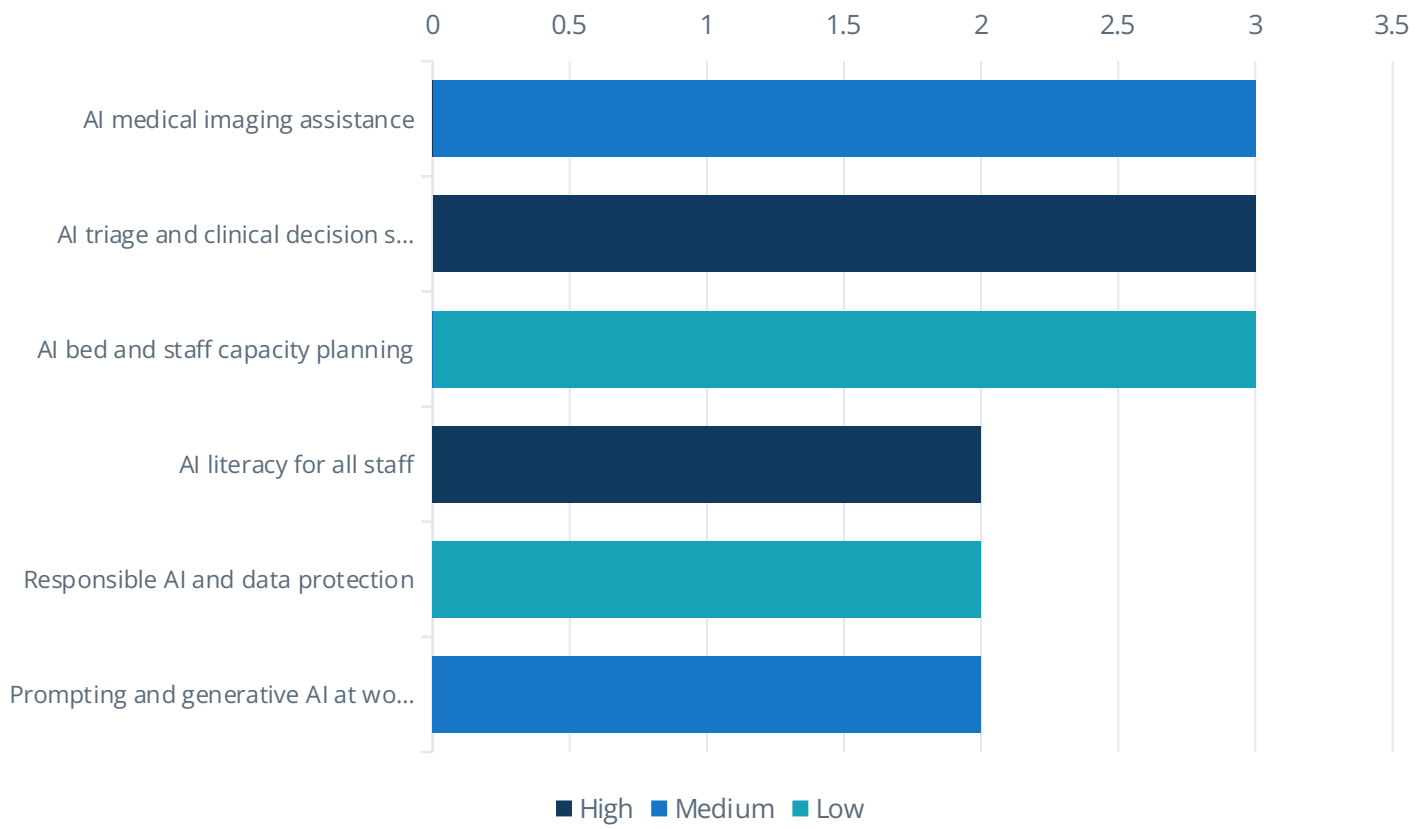
12 · 0 · 0

confirmed · lowered · raised by the manager

Validation by the manager is what turns a self-assessment into a measured gap: only validated levels feed the skill gaps and the needs register.

15 requests received, all traced to a theme

Individual and collective requests from managers and employees (E2E-27)



Status	Requests
Planned	4
Validated	7
Rejected	4

Type	Requests
Individual	11
Collective	4

Requests by theme and priority.

Clinical Care & Nursing

Needs, assessment and competences of the function that carries the strategic objective SO-1

Decision level	Score (%)	Status	Gap
MS	59 %	Red	AI triage and clinical decision support
MO	56 %	Red	AI medical imaging assistance
OP	64 %	Red	AI bed and staff capacity planning

Respondent	Population	Strengths / Areas for improvement	Competences
Reda Naciri	Management	Soins cliniques et infirmiers — Good knowledge of the market — Little…	AI bed and staff capacity planning
Reda Naciri	Management	Clinical Care & Nursing — Good knowledge of the market — Reporting ke…	AI bed and staff capacity planning; Time management and priorities
Zineb Hajji	Team member	Experienced, committed team — Low use of AI medical imaging assistance	AI medical imaging assistance
Zineb Hajji	Team member	Stable processes — Low use of AI medical imaging assistance	ISO 7101 requirements; Dashboards
Ayoub Benjelloun	Management	Soins cliniques et infirmiers — Stable processes — Little versatility…	AI bed and staff capacity planning
Ayoub Benjelloun	Management	Clinical Care & Nursing — Good knowledge of the market — Reporting ke…	ISO 7101 requirements; Dashboards

Source: TER sections 3.36 to 3.39.

How the teams use AI and what holds them back

Current use

Daily, for triage and reports. (33)

Barriers

Lack of training and unclear rules. (33)

Risks to manage

Data protection and validation of results. (33)

Priorities

AI bed and staff capacity planning; data quality (12)

AI medical imaging assistance; data quality (11)

Preferred format

Short workshops on the job, then coaching. (33)

Strategic planning practice	Yes	No
Is there a strategic plan with macro business plans at company level?	1	0
Are this strategic plan and the macro business plans implemented?	1	0
Are they monitored?	0	1
Are they subject to continuous improvement?	0	1

Most frequent answers of 37 respondents (TER 3.45, 3.46).

What the respondents want the training to change

Objective	SMART formulation	Mentions
Improve customer satisfaction	Satisfaction score ≥ 85% by end of year	30
Reduce non-quality costs	-20% rework in 12 months	30
Develop human capital	80% of critical roles certified	14

Expected results

- Teams able to use AI triage and clinical decision support safely.
- Better audit results and staff retention.
- Fewer errors and faster service.
- Teams able to use AI medical imaging assistance safely.

Source: TER 3.42 (74 objectives stated) and 3.9.

03

Training Engineering Report

134 sections, 198 pages, generated from the run data

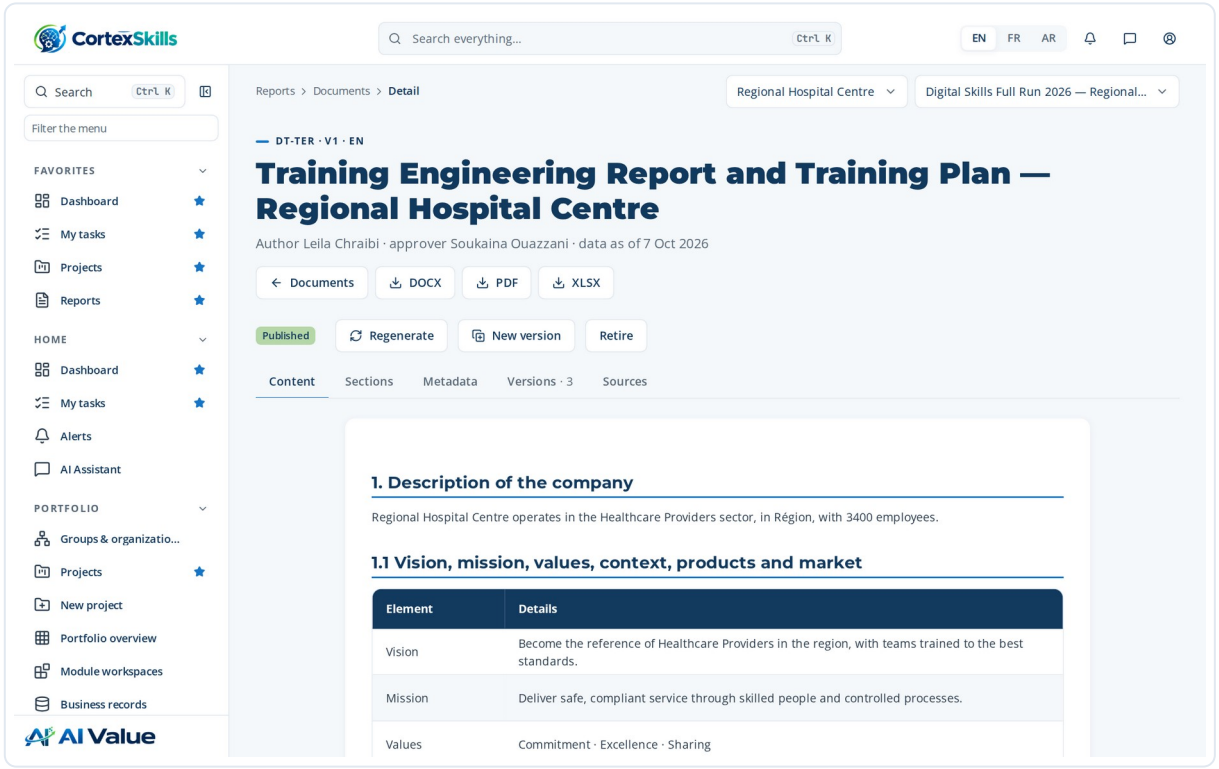
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Training Engineering Report and Training Plan

Version 1 · author Leila Chraibi · approved by Soukaina Ouazzani

Chapter	Sections
1. Description of the company	14
2. The previous training plan	2
3. Competence needs	50
4. Perspectives	3
5. The prioritized training plan	23
6. Risks, controls and indicators	2
7. Annexes — interview forms	38

Plus identification, data sources and approval.



The report in the document editor.

Needs linked to the strategy and to the HR orientations

Priority axes named by the General Management: Leadership and performance · AI triage and clinical decision support · ISO 7101 compliance · Dashboards

ID	Objective	Horizon
SO-1	Adopt AI triage and clinical decision support across Clinical Care & Nursing by end 2027	2027-12-31
SO-2	Zero major findings in ISO 7101 audits	2026-12-31

HR orientations

• Update job descriptions, individual objectives and the yearly assessment; build career paths.

Change management

1) Focus and prioritization · 2) Stay positive · 3) Persevere and follow up with short daily repetitions · Patience · Lead by example · Progressiveness.

Source: TER sections 3.1, 3.5 and 3.6.

Four initiatives deployed in three waves

The perspectives take into account the strategic orientations, the SWOT and improvement axes, the HR orientations and the recommendations made in the face-to-face meetin...

Wave 1

- AI triage and clinical decision support — impact 5
- AI literacy for all staff — impact 5

Wave 2

- AI medical imaging assistance — impact 4

Wave 3

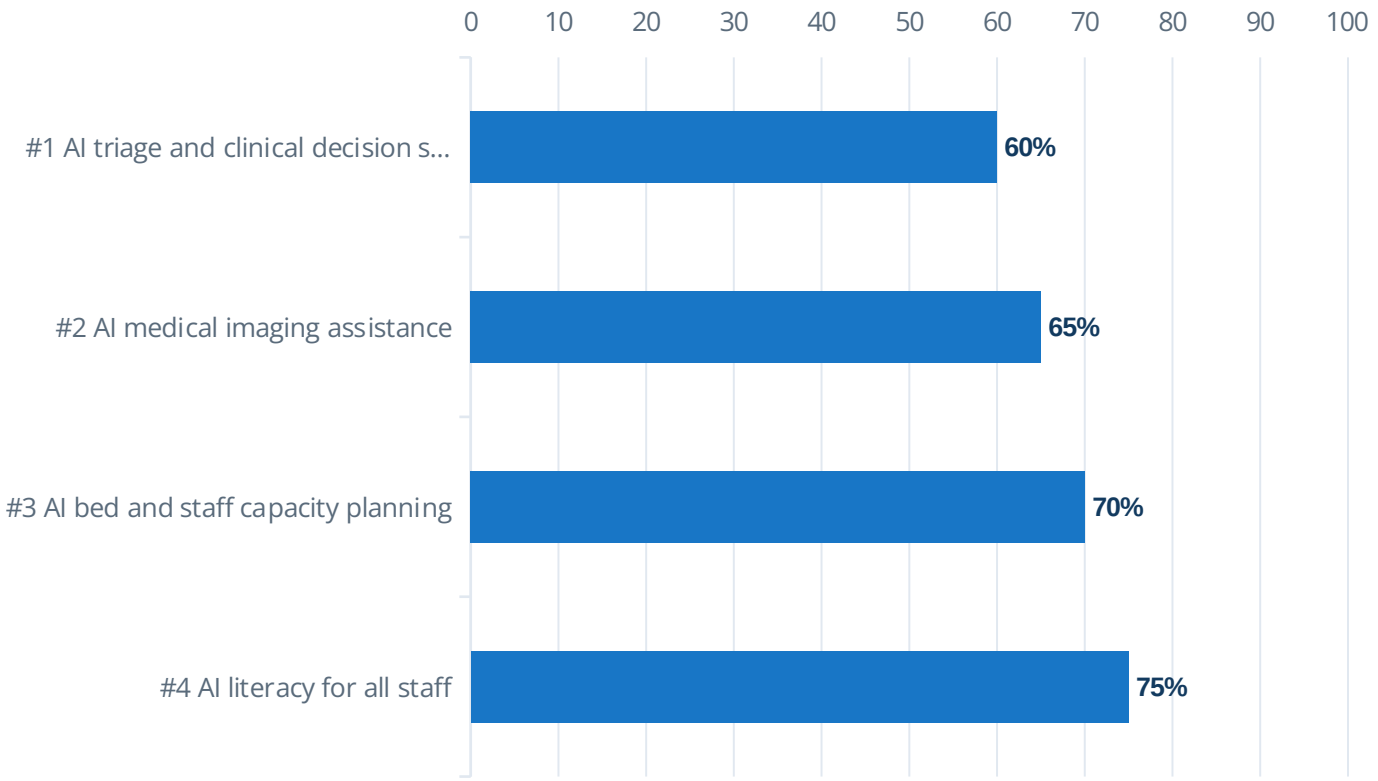
- AI bed and staff capacity planning — impact 3

#	Improvement axis	Implemented	Wave
1	AI triage and clinical decision support	60%	1
2	AI medical imaging assistance	65%	2
3	AI bed and staff capacity planning	70%	3
4	AI literacy for all staff	75%	1

Source: TER chapter 4 and section 4.1.

Each axis ranked, with its implementation progress

Improvement axes drawn from the SWOTs and followed in the roadmap



Roadmap initiative	Wave
AI triage and clinical decision support	1
AI literacy for all staff	1
AI medical imaging assistance	2
AI bed and staff capacity planning	3

Average implementation 68 %: wave 1 axes are deployed, the others are carried by waves 2 and 3.

Implementation of each improvement axis, by priority rank.

Each risk reduced by named controls

ID	Risk or opportunity	Category	Inherent	Residual	Controls
RISK-014	Low learner and user adoption	Adoption	16	8	CTRL-081
RISK-017	Critical knowledge lost with expert departure	Operational	16	8	CTRL-070; CTRL-082
RISK-001	Unauthorized access to tenant data	Security	20	6	CTRL-001; CTRL-002; CTRL-048; CTRL-011
RISK-005	Training plan misaligned with strategy	Strategic	16	6	CTRL-049; CTRL-057; CTRL-076
RISK-006	Inaccurate skills data leading to wrong needs	Data quality	15	6	CTRL-064; CTRL-045
RISK-030	Position descriptions diverge from reality	Organizational	12	6	CTRL-066
RISK-007	Unverified AI content in the Training Engineering Report	AI	16	5	CTRL-041; CTRL-056
RISK-009	Expired mandatory certification	Compliance	20	5	CTRL-053; CTRL-054
RISK-031	Site deployment deviates from group plan	Operational	12	5	CTRL-071

Top 9 of 35 risks by residual score (TER chapter 6).

Indicators followed every month during the run

ID	Indicator	Target	Latest value	Status
KPI-003	Critical gap rate	≤ 10%	2026-10: 8.2	Green
KPI-004	Training Engineering Report cycle time	≤ 30 days	2026-10: 24.6	Green
KPI-005	Design approval lead time	≤ 20 days	2026-10: 16.4	Green
KPI-007	Content publication lead time	≤ 15 days	2026-10: 12.3	Green
KPI-008	Session fill rate	≥ 85%	2026-10: 91.8	Green
KPI-009	Plan realization rate	≥ 90%	2026-10: 97.2	Green
KPI-010	Learner completion rate	≥ 80%	2026-10: 86.4	Green
KPI-012	Average Level 1 satisfaction	≥ 4.2	2026-10: 4.5	Green
KPI-013	Skill progression rate	≥ 70%	2026-10: 75.6	Green
KPI-015	Training record correction rate	≤ 1%	2026-10: 0.82	Green

10 of the 61 indicators of TER section 6.1.

04

Training Plan

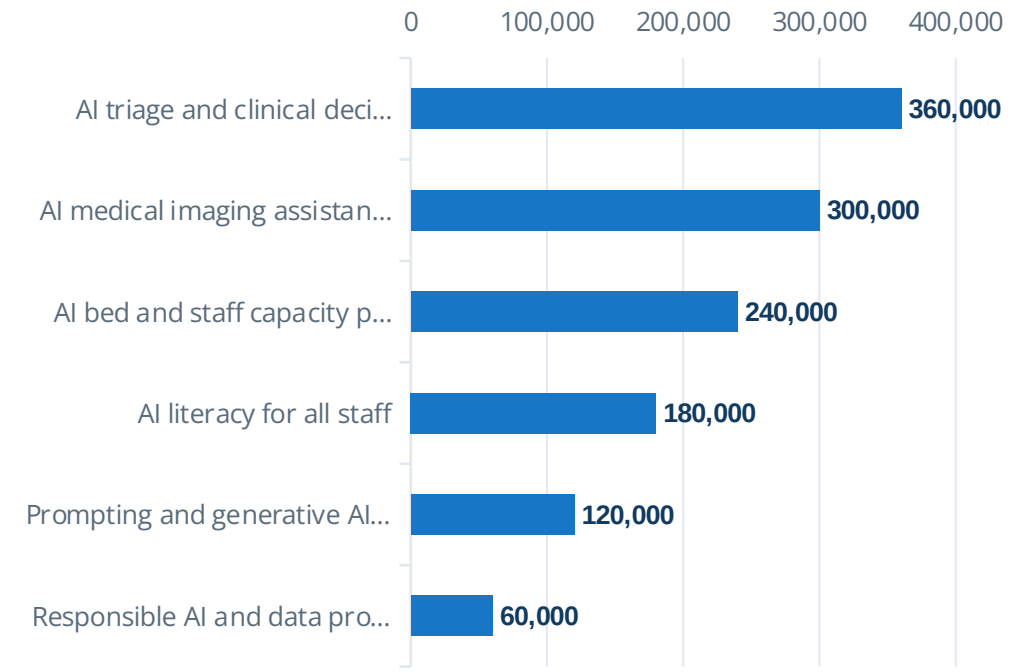
The plan 2026 holds 2 programs and 6 trainings, 12 days per group in total and 44 group-days to deliver, for a budget of 1,260,000.

AI Value
Digital & AI Transformation

Six themes ranked by impact and urgency

Plan 2026 — status Locked — budget 1,260,000.

#	Theme	Days	Groups	Budget
1	AI triage and clinical decision support	3	6	360,000
2	AI medical imaging assistance	2	4	300,000
3	AI bed and staff capacity planning	2.5	3	240,000
4	AI literacy for all staff	1.5	3	180,000
5	Prompting and generative AI at work	1	2	120,000
6	Responsible AI and data protection	2	2	60,000



Budget by theme.

Two programs, six trainings, clear prerequisites

Code	Program	Strategic axis	Trainings
PRG-1	AI skills for Clinical Care & Nursing	Execution of the core function	TR-AI-01, TR-AI-02, TR-AI-03
PRG-2	AI foundations for all staff	Leadership, performance and responsible use	TR-AI-04, TR-AI-05, TR-AI-06

ID	Training	Level	Duration	Prerequisites
TR-AI-01	AI triage and clinical decision support	Intermediate	3 day(s)	Basic digital literacy; the Foundation training is recommended
TR-AI-02	AI medical imaging assistance	Intermediate	2 day(s)	Basic digital literacy; the Foundation training is recommended
TR-AI-03	AI bed and staff capacity planning	Advanced	2.5 day(s)	TR-AI-01 or equivalent experience
TR-AI-04	AI literacy for all staff	Foundation	1.5 day(s)	None — open to all staff
TR-AI-05	Prompting and generative AI at work	Foundation	1 day(s)	None — open to all staff
TR-AI-06	Responsible AI and data protection	Intermediate	2 day(s)	Basic digital literacy; the Foundation training is recommended

Source: Training Plan "Programs" and TER section 5.1.

TRAINING SHEET

TR-AI-01 — AI triage and clinical decision support

Level Intermediate · 3 day(s) · program PRG-1

Learning objectives

• Explain the key concepts of AI triage and clinical decision support and how they apply to Clinical Care & Nursing
• Apply the methods and tools of AI triage and clinical decision support to a real case from Regional Hospital Centre
• Plan the deployment of AI triage and clinical decision support with its indicators and controls (ISO 7101)

Prerequisites

Basic digital literacy; the Foundation training is recommended

Delivery

M1 AI triage and clinical decision support · E-learning · 2 h
M3 AI triage and clinical decision support · Virtual classroom · 3.5 h

DETAILED AGENDA

AI triage and clinical decision support — half-day by half-day

22 activities over 3 day(s): lectures, quizzes and workshops on the hospital's own cases.

Day 1 — morning

Lecture · Foundations of AI triage and clinical decision support: key c... (75 min)
Lecture · Why AI triage and clinical decision support matters for Clini... (45 min)
Quiz — key concepts of AI triage and clinical decision support (20 min)
Workshop — map today's practices in Clinical Care & Nursing a... (75 min)

Day 1 — afternoon

Lecture · Methods and tools of AI triage and clinical decision support (75 min)
Lecture · Worked example from the Healthcare Providers sector (40 min)
Quiz — choosing the right method (20 min)
Workshop — apply the method to a real case from Regional Hosp... (80 min)

Day 2 — morning

Lecture · Governance, risks and ISO 7101 requirements (60 min)
Lecture · Data, quality and evidence to keep (45 min)
Quiz — compliance and risk scenarios (20 min)
Workshop — build the control checklist of your process (80 min)

Day 2 — afternoon

Lecture · Deploying AI triage and clinical decision support: roles, cha... (60 min)
Lecture · Indicators that measure the results (45 min)
Quiz — adoption and indicator cases (20 min)
Workshop — design the indicator dashboard of your team (80 min)

Day 3 — morning

Lecture · Advanced practices and lessons learned in Clinical Care & Nur... (75 min)
Quiz — advanced cases (25 min)
Workshop — solve a critical incident scenario (100 min)

Day 3 — afternoon

Lecture · Transfer to the workplace: the 30-60-90-day plan (60 min)
Quiz · Final assessment (Kirkpatrick level 2) (30 min)
Workshop — individual action plan and peer review (90 min)

Why each audience should take the training

Training TR-AI-01 — behaviour, pain points and hopes of each persona

Executive sponsor — General Manager

Decides on facts presented in short steering meetings; delegates execution to the management team; follows a few indicators.

Training spend with no visible link to the strategic objectives; results of the previous plan hard to prove; ISO 7101 audits that expose competence gaps.

A prioritized plan that serves the strategy, a measurable return, and a management team able to lead the change.

Manager — director, head of department, process owner

Balances daily operations and improvement projects; prefers practical, short formats; relies on key people in the team.

Team absent from the job during long trainings; skills gaps on critical activities of Clinical Care & Nursing; processes not documented or without an owner.

Trainings that solve the function's real problems, applied on the job the following week, with indicators the manager can follow.

Team member — Clinical Care & Nursing

Learns by doing and from peers; uses a phone more than a computer; wants to know what changes in daily work.

New tools introduced without explanation; theoretical trainings far from the tasks; fear of making mistakes with AI triage and clinical decision support.

Practical skills recognized by a certificate, less rework, and a clear path to the next position.

TRAINING SHEET

TR-AI-02 — AI medical imaging assistance

Level Intermediate · 2 day(s) · program PRG-1 · 4 half-days, 15 activities, 14 h

Learning objectives

• Explain the key concepts of AI medical imaging assistance and how they apply to Clinical Care & Nursing
• Apply the methods and tools of AI medical imaging assistance to a real case from Regional Hospital Centre
• Plan the deployment of AI medical imaging assistance with its indicators and controls (ISO 7101)

Prerequisites and activity mix

Basic digital literacy; the Foundation training is recommended
Lecture: 7 · Quiz: 4 · Workshop: 4

Half-day agenda

Half-day	Activities	min
Day 1 — morning	Foundations of AI medical imaging assistance: key concepts and vocabulary · Why AI medical imaging assistance matters f…	215
Day 1 — afternoon	Methods and tools of AI medical imaging assistance · Worked example from the Healthcare Providers sector · Quiz — choos…	215
Day 2 — morning	Governance, risks and ISO 7101 requirements · Data, quality and evidence to keep · Quiz — compliance and risk scenarios…	205
Day 2 — afternoon	Transfer to the workplace: the 30-60-90-day plan · Final assessment (Kirkpatrick level 2) · Workshop — individual actio…	180

Personas: Executive sponsor — General Manager · Manager — director, head of department, process owner · Team member — Clinical Care & Nursing

TRAINING SHEET

TR-AI-03 — AI bed and staff capacity planning

Level Advanced · 2.5 day(s) · program PRG-1 · 5 half-days, 19 activities, 17 h

Learning objectives

• Explain the key concepts of AI bed and staff capacity planning and how they apply to Clinical Care & Nursing
• Apply the methods and tools of AI bed and staff capacity planning to a real case from Regional Hospital Centre
• Plan the deployment of AI bed and staff capacity planning with its indicators and controls (ISO 7101)

Prerequisites and activity mix

TR-AI-01 or equivalent experience
Lecture: 9 · Quiz: 5 · Workshop: 5

Half-day agenda

Half-day	Activities	min
Day 1 — morning	Foundations of AI bed and staff capacity planning: key concepts and vocabulary · Why AI bed and staff capacity planning…	215
Day 1 — afternoon	Methods and tools of AI bed and staff capacity planning · Worked example from the Healthcare Providers sector · Quiz — …	215
Day 2 — morning	Governance, risks and ISO 7101 requirements · Data, quality and evidence to keep · Quiz — compliance and risk scenarios…	205
Day 2 — afternoon	Deploying AI bed and staff capacity planning: roles, change and adoption · Indicators that measure the results · Quiz —…	205
Day 3 — morning	Transfer to the workplace: the 30-60-90-day plan · Final assessment (Kirkpatrick level 2) · Workshop — individual actio…	180

Personas: Executive sponsor — General Manager · Manager — director, head of department, process owner · Team member — Clinical Care & Nursing

TRAINING SHEET

TR-AI-04 — AI literacy for all staff

Level Foundation · 1.5 day(s) · program PRG-2 · 3 half-days, 11 activities, 10 h

Learning objectives

• Explain the key concepts of AI literacy for all staff and how they apply to Clinical Care & Nursing
• Apply the methods and tools of AI literacy for all staff to a real case from Regional Hospital Centre
• Plan the deployment of AI literacy for all staff with its indicators and controls (ISO 7101)

Prerequisites and activity mix

None — open to all staff
Lecture: 5 · Quiz: 3 · Workshop: 3

Half-day agenda

Half-day	Activities	min
Day 1 — morning	Foundations of AI literacy for all staff: key concepts and vocabulary · Why AI literacy for all staff matters for Clini…	215
Day 1 — afternoon	Methods and tools of AI literacy for all staff · Worked example from the Healthcare Providers sector · Quiz — choosing …	215
Day 2 — morning	Transfer to the workplace: the 30-60-90-day plan · Final assessment (Kirkpatrick level 2) · Workshop — individual actio…	180

Personas: Executive sponsor — General Manager · Manager — director, head of department, process owner · Team member — Clinical Care & Nursing

TRAINING SHEET

TR-AI-05 — Prompting and generative AI at work

Level Foundation · 1 day(s) · program PRG-2 · 2 half-days, 7 activities, 7 h

Learning objectives

• Explain the key concepts of Prompting and generative AI at work and how they apply to Clinical Care & Nursing
• Apply the methods and tools of Prompting and generative AI at work to a real case from Regional Hospital Centre
• Plan the deployment of Prompting and generative AI at work with its indicators and controls (ISO 7101)

Prerequisites and activity mix

None — open to all staff
Lecture: 3 · Quiz: 2 · Workshop: 2

Half-day agenda

Half-day	Activities	min
Day 1 — morning	Foundations of Prompting and generative AI at work: key concepts and vocabulary · Why Prompting and generative AI at wo…	215
Day 1 — afternoon	Transfer to the workplace: the 30-60-90-day plan · Final assessment (Kirkpatrick level 2) · Workshop — individual actio…	180

Personas: Executive sponsor — General Manager · Manager — director, head of department, process owner · Team member — Clinical Care & Nursing

TRAINING SHEET

TR-AI-06 — Responsible AI and data protection

Level Intermediate · 2 day(s) · program PRG-2 · 4 half-days, 15 activities, 14 h

Learning objectives

• Explain the key concepts of Responsible AI and data protection and how they apply to Clinical Care & Nursing
• Apply the methods and tools of Responsible AI and data protection to a real case from Regional Hospital Centre
• Plan the deployment of Responsible AI and data protection with its indicators and controls (ISO 7101)

Prerequisites and activity mix

Basic digital literacy; the Foundation training is recommended
Lecture: 7 · Quiz: 4 · Workshop: 4

Half-day agenda

Half-day	Activities	min
Day 1 — morning	Foundations of Responsible AI and data protection: key concepts and vocabulary · Why Responsible AI and data protection…	215
Day 1 — afternoon	Methods and tools of Responsible AI and data protection · Worked example from the Healthcare Providers sector · Quiz — …	215
Day 2 — morning	Governance, risks and ISO 7101 requirements · Data, quality and evidence to keep · Quiz — compliance and risk scenarios…	205
Day 2 — afternoon	Transfer to the workplace: the 30-60-90-day plan · Final assessment (Kirkpatrick level 2) · Workshop — individual actio…	180

Personas: Executive sponsor — General Manager · Manager — director, head of department, process owner · Team member — Clinical Care & Nursing

Paths, modules and content assets

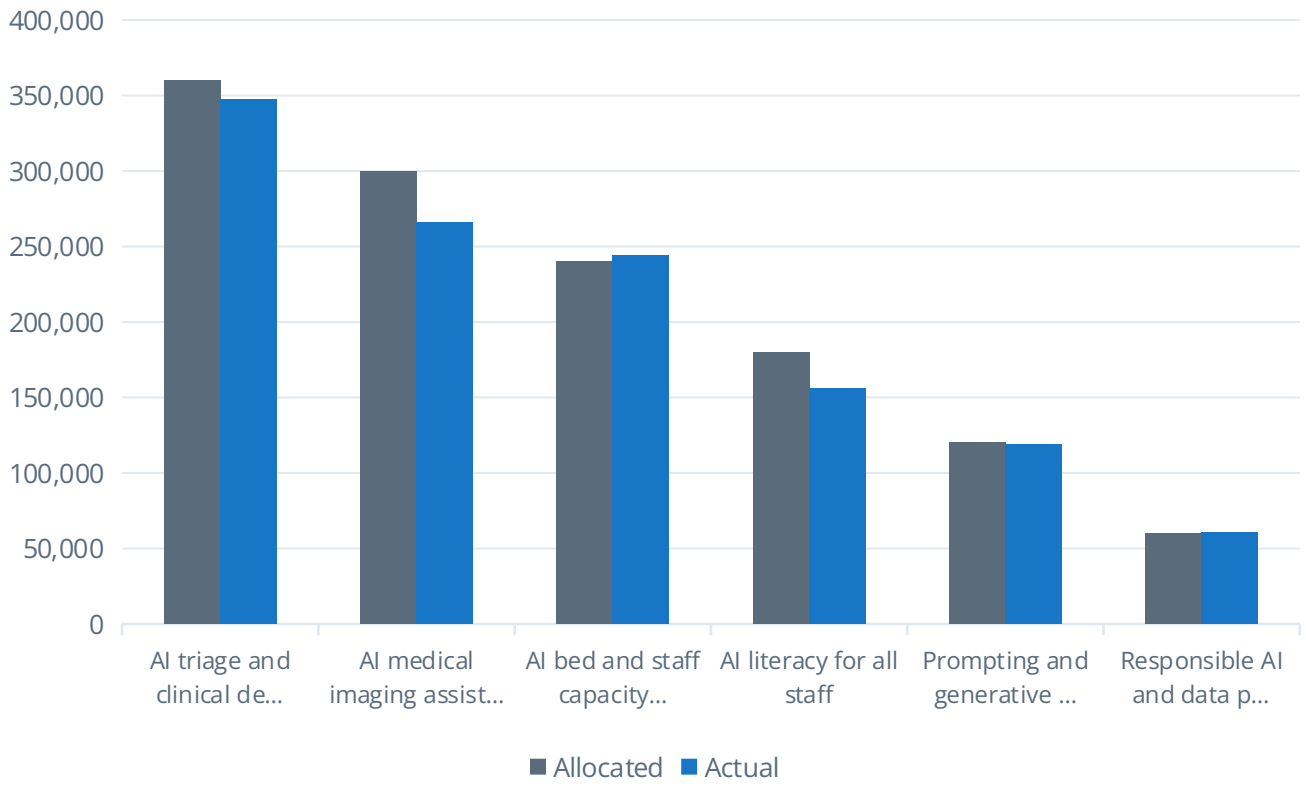
Learning path	Days	Module	Modality	Hours	Design
AI triage and clinical decision support	3	M1 AI triage and clinical decision support	E-learning	2	Approved
		M3 AI triage and clinical decision support	Virtual classroom	3.5	Approved
AI medical imaging assistance	2	M2 AI medical imaging assistance	Classroom	7	Approved
		M4 AI medical imaging assistance	Classroom	7	Approved

Content	Format	Size (MB)	Review date	Status	Language
M1 material	SCORM	42	2027-08-03	Published	EN
M2 material	Video	180	2027-04-05	Published	FR
M3 material	PDF	3.2	2026-12-06	Published	AR
M4 material	Capsule	12	2026-08-08	Published	FR

Source: TER sections 5.3 and 5.4.

Allocated, committed, spent and refunds expected

Allocated 1,260,000, committed 1,260,000 (100 %), spent 1,192,662 (95 % of planned); expected refunds 352,800.



Allocated (Muted) versus actual (Azure) by training, (TER 5.7).

Line	Actual	Refund
AI triage and clinical decisi...	347,127	100,800
AI medical imaging assistance	266,091	84,000
AI bed and staff capacity pla...	243,800	67,200
AI literacy for all staff	156,192	50,400
Prompting and generative AI a...	118,913	33,600
Responsible AI and data prote...	60,539	16,800
Total	**1,192,662**	352,800

Provider	Accredited until	/5
Atlas Formation Conseil	2027-04-25	4.2
Maghreb Learning Partners	2027-08-03	3.8

05

Delivery, evaluation and audit

Sessions, Kirkpatrick levels, ROI, internal audit and lessons learned

AI Value
Digital & AI Transformation

Four sessions delivered with their enrolments

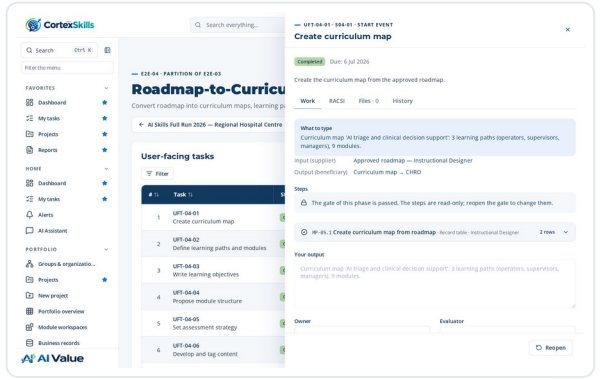
Session	Date	Capacity	Trainer	Enrolled
S-01 AI triage and clinical decision supp…	2026-07-19	12	Rachid Benjelloun	3
S-02 AI medical imaging assistance	2026-07-26	12	Rachid Benjelloun	3
S-03 AI triage and clinical decision supp…	2026-08-02	12	Rachid Benjelloun	3
S-04 AI medical imaging assistance	2026-08-09	12	Rachid Benjelloun	3

10 / 12

learners present at their session

10 / 12

enrolments completed



A task of the delivery process with its typed steps.

EVALUATION

Results measured at three Kirkpatrick levels

86 %
1 — Reaction

93 %
2 — Learning

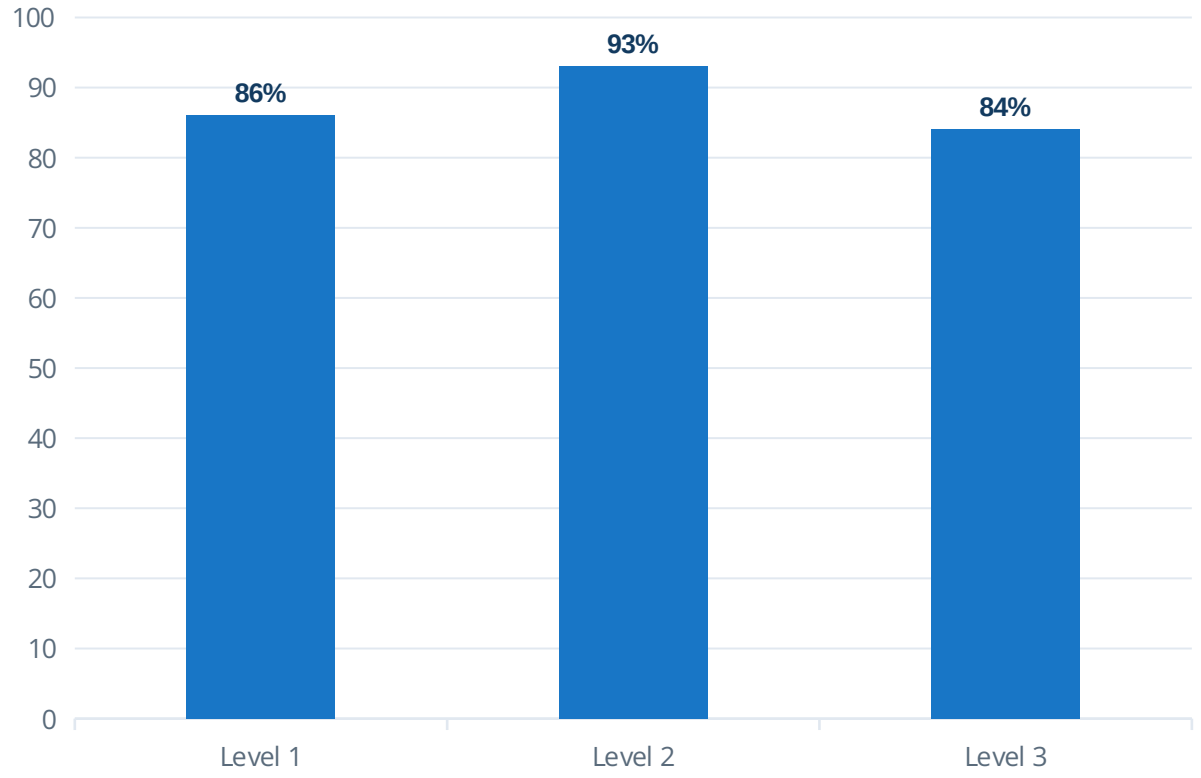
84 %
3 — Behaviour

Level	What is measured	When	Instrument	Results
1 — Reaction	Satisfaction and perceived usefulness	End of each session	Satisfaction questionnaire	2 × 86 %
2 — Learning	Knowledge and skills acquired	End of the training	Pre- and post-test, case study	2 × 93 %
3 — Behaviour	Use of the skills at the workstation	Three months after	Manager observation grid	2 × 84 %
4 — Results	Effect on the indicators of the function	Six to twelve months after	Indicators of the function (KPI)	—

Certifications (TER 5.10): 4 valid, 1 expiring, 1 expired — the expired one triggers an alert and a renewal session.

Each evaluation recorded with its score

6 evaluations linked to the enrolments of the sessions (E2E-07)



Average score by Kirkpatrick level.

Evaluation	Level	Score
Kirkpatrick level 1	1	93 %
Kirkpatrick level 1	1	79 %
Kirkpatrick level 2	2	93 %
Kirkpatrick level 2	2	92 %
Kirkpatrick level 3	3	76 %
Kirkpatrick level 3	3	92 %

Certifications followed, renewals planned before expiry

Regulatory and accreditation requirements per employee (E2E-09)

4
valid certifications

1
expiring within 90 days: renewal session planned

1
expired: alert raised to the manager

Certification	Requirement	Valid until	Status
ISO 7101 awareness	ISO 7101	2026-08-18	Valid
ISO 7101 awareness	ISO 7101	2026-11-16	Expired
ISO 7101 awareness	ISO 7101	2027-02-14	Valid
ISO 7101 awareness	ISO 7101	2027-05-15	Expiring
ISO 7101 awareness	ISO 7101	2027-08-13	Valid
ISO 7101 awareness	ISO 7101	2027-11-11	Valid

Phillips ROI and the indicators of the ROI study

ROI and business impact report — Regional Hospital Centre

108
Training ROI

4.5
Average Level 1 satisfaction

75.6
Skill progression rate

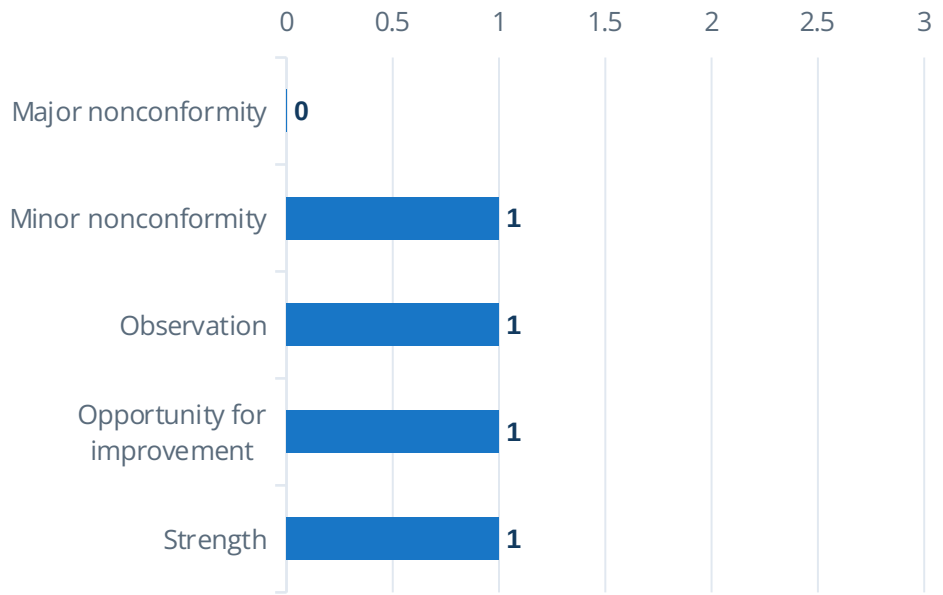
4.1
Budget execution variance

ID	Indicator	Formula	Target	Latest value	Status
KPI-037	Training ROI	(Net programme benefits / programme costs) x 100 (Phillips)	≥ 100%	108 (2026-10)	Green
KPI-012	Average Level 1 satisfaction	Mean reaction score across closed sessions (1-5)	≥ 4.2	4.5 (2026-10)	Green
KPI-013	Skill progression rate	Learners gaining at least one level within 6 months / learners traine…	≥ 70%	75.6 (2026-10)	Green
KPI-022	Budget execution variance	|Actual - allocated| / allocated x 100	≤ 5%	4.1 (2026-10)	Green
KPI-023	OFPPT refund recovery rate	Refunds received / refunds forecast x 100	≥ 90%	97.2 (2026-10)	Green

Source: ROI and business impact report (DT-E2E-08), indicators section.

Audit report — Regional Hospital Centre

Conforming with one minor nonconformity to correct within 60 days.



Findings by grade.

ID	Grade	Requirement	Statement
F-1	Minor nonconformity	ISO 7101 7.2	Two trainers have no evidence of competence on file.
F-2	Observation	ISO 7101 9.1	Level 3 evaluations are planned but not yet scheduled.
F-3	Opportunity for improvement	ISO 7101 7.1.6	Capture lessons learned at the end of each session.
F-4	Strength	ISO 7101 7.2	Needs analysis fully traced from strategic objectives.

Strengths: Clear needs analysis; strong management involvement; complete attendance records.
Follow-up: Verification of the corrective action at the next steering committee.

ACTION PLAN

Corrective actions owned, dated and evaluated

Actions raised by the audit and the indicators, each with an owner and an evaluator (governance)

Action	Due date	Status
Update 11 outdated position descriptions	2026-07-09	Open
Contract a second training vendor	2026-06-09	Done

How actions are governed

An owner carries the action, a different person evaluates its effectiveness.

Overdue actions raise an alert to the owner and the project manager.

Closure needs the evaluation: the audit finding is closed only then.

REX — AI triage and clinical decision support

What went well

The stakeholder register and multi-channel distribution lifted the response rate.

What did not

SWOT inputs arrived late from one function.

Root cause

No backup nominated for the function head.

What we will do next time

Name a backup for every function head at kickoff.

06

Documents produced

37 documents published in English, each traced to its data

AI Value
Digital & AI Transformation

Every end-to-end process leaves its document

All published, versioned and approved by a second person — 1,060 pages.

Code	Document	Pages
DT-E2E-01	Annual L&D plan and budget	28
DT-E2E-02	Skills gap diagnosis	23
DT-E2E-03	Training needs report and roadmap	28
DT-E2E-04	Curriculum design document	24
DT-E2E-05	Training delivery report	28
DT-E2E-06	Learner engagement report	30
DT-E2E-07	Training evaluation report	23
DT-E2E-08	ROI and business impact report	23
DT-E2E-09	Compliance and certification register	22
DT-E2E-10	Internal mobility and career paths	25

Code	Document	Pages
DT-E2E-11	Onboarding programme record	21
DT-E2E-12	Knowledge capture and transfer record	22
DT-E2E-13	Content catalogue and lifecycle	22
DT-E2E-14	Extended enterprise training report	22
DT-E2E-15	Data, AI and integration governance	24
DT-E2E-16	Vendor management file	20
DT-E2E-17	Change and adoption plan	23
DT-E2E-18	Quality, risk and improvement report	30
DT-E2E-19	Consulting engagement closure report	25
DT-E2E-20	Multi-axis diagnostic and maturity report	26

Documents produced (continued)

Code	Document	Pages
DT-E2E-21	Previous plan impact evaluation	25
DT-E2E-22	Multi-site deployment plan	24
DT-E2E-23	Regulatory submission file	22
DT-E2E-24	Competitive intelligence note	23
DT-E2E-25	Job architecture and competency map	23
DT-E2E-26	Forward planning of jobs and skills (GPEC)	24
DT-E2E-27	Training demand consolidation	21
DT-E2E-28	Performance assessment and RAG dashboard	22
DT-E2E-29	Functional SWOT and improvement axes	23
DT-E2E-30	Prioritized resource plan	24

Code	Document	Pages
DT-E2E-31	Post-engagement roadmap	24
DT-E2E-32	Scope of work and stakeholder register	31
DT-E2E-33	Questionnaire campaign report	29
DT-TER	Training Engineering Report and Training Plan	198
DT-PLAN	Training plan with detailed agendas	34
DT-AUDIT	Audit report	5
DT-MASTER	Master list of documented information	19

How the teams entered and governed the data

CortexSkills 1.11 — Standard Application SRS v1.8

Typed step forms

Each step asks for what it produces: records, matrices, decisions; rows are saved one by one, with Undo.

Full-screen entry

Any table opens over the whole screen with zoom (50–200 %), add, save and return to the same row.

Paste from Excel or Word

Lists of staff or trainings are pasted in one step, checked cell by cell, saved and undone as one operation.

Amounts with their currency

Budgets carry their currency; totals never mix currencies without a stated exchange rate and date.

Two-person publication

A document is published only once a second person approves it; every version is kept.

Audit trail

Every change, paste and approval is recorded with its author, date and justification.

TRACEABILITY

Every number of the report comes from a record

Every table of this document is drawn from the application's records; data as of 07/10/2026, 15:28:07.

Source	Records	Process	Last change
Questionnaire responses	32	MP-54	—
Stakeholder	40	MP-53	2026-10-07
ScopeCriterion	17	MP-52	2026-10-07
SkillAssessment	12	MP-16	2026-10-07
SkillGap	8	MP-16	2026-10-07
TrainingNeed	10	MP-03	2026-10-07
RoadmapInitiative	4	MP-04	2026-10-07
WbsNode	3	MP-49	2026-10-07
Session	4	MP-05	2026-10-07
TrainingPlan	1	MP-49	2026-10-07
Employee	16	MP-16	2026-10-07
StrategicObjective	2	MP-01	2026-10-07

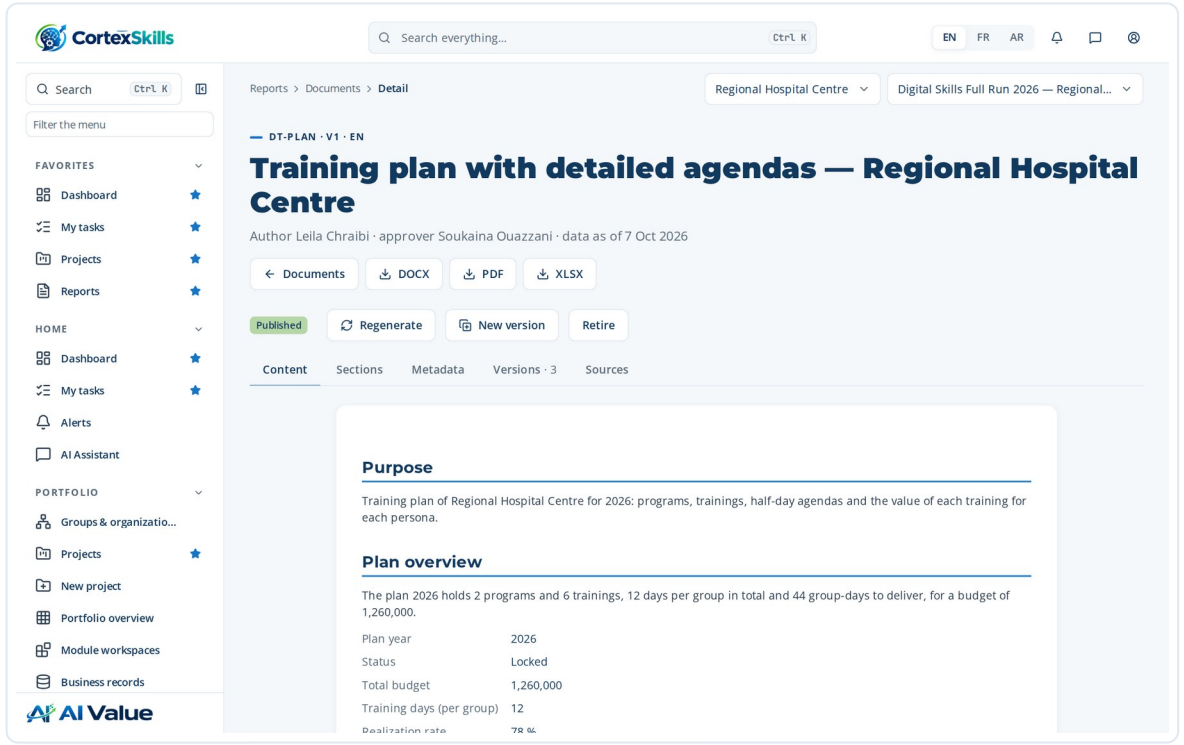
Governance of the documents

Author Leila Chraibi, approver Soukaina Ouazzani: two different people.

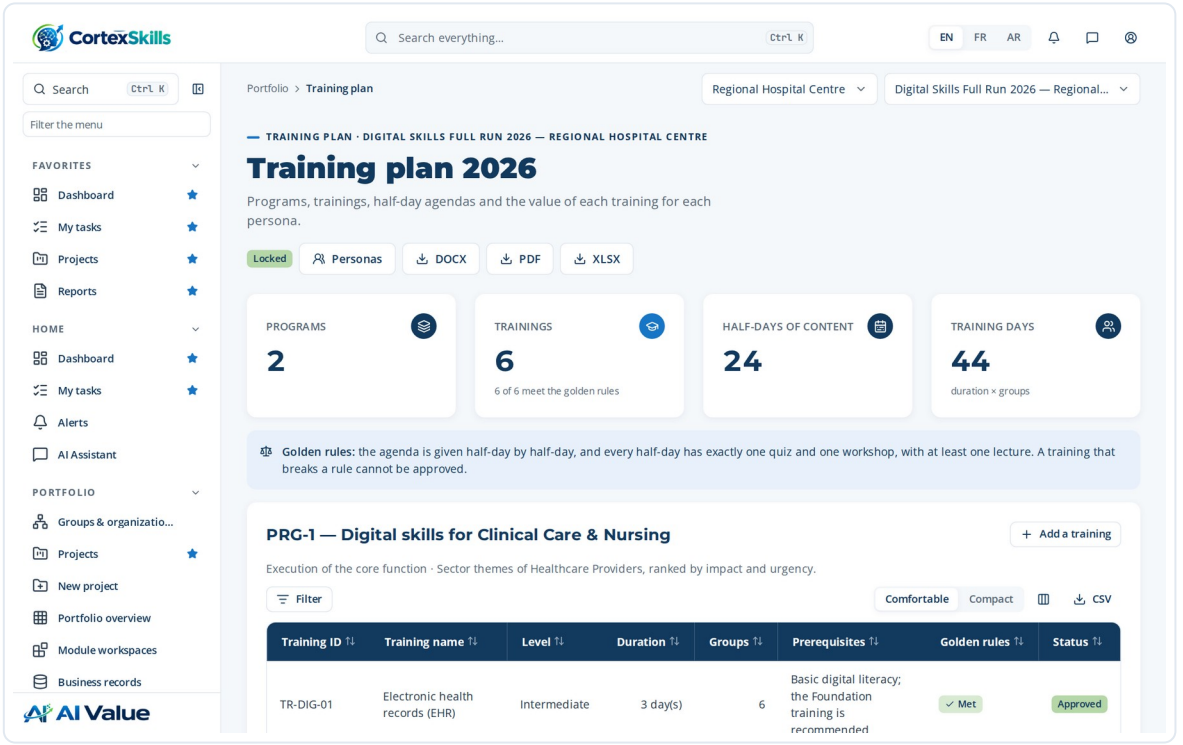
Each version is kept; a change of the data marks the document as stale.

20 data sources listed with their record counts and process.

The Training Plan as the users see it

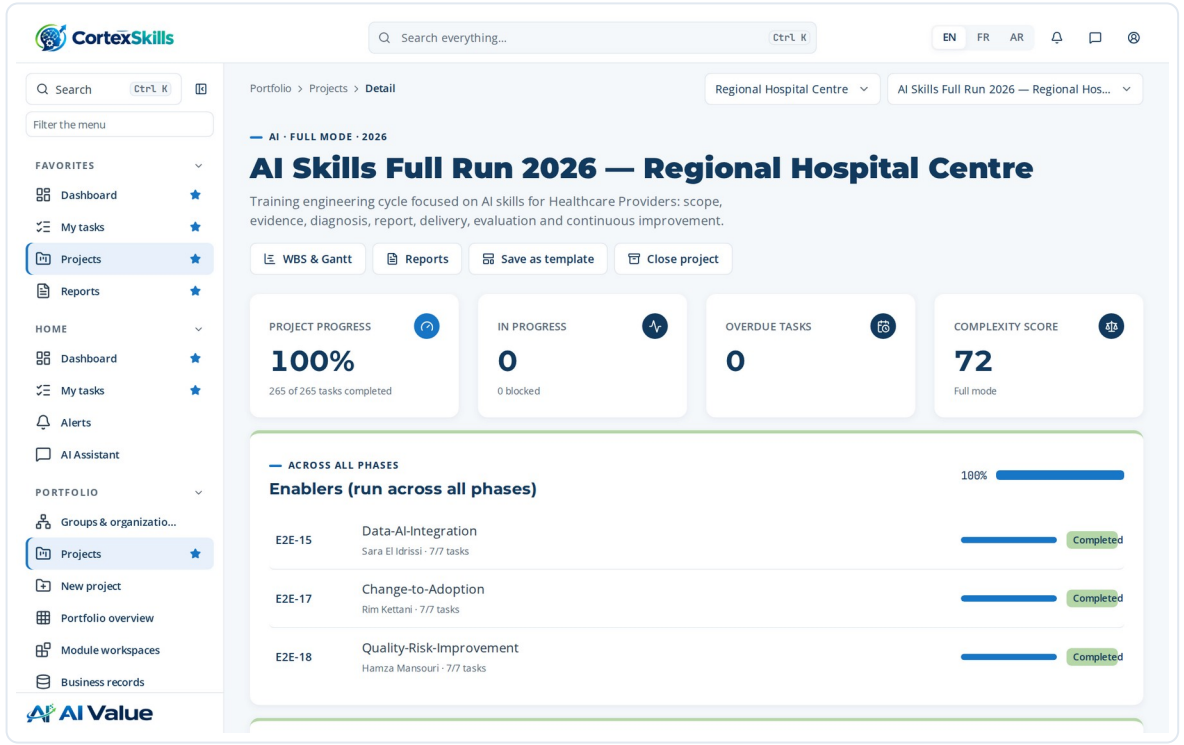


Training plan with detailed agendas — Regional Hospital Centre

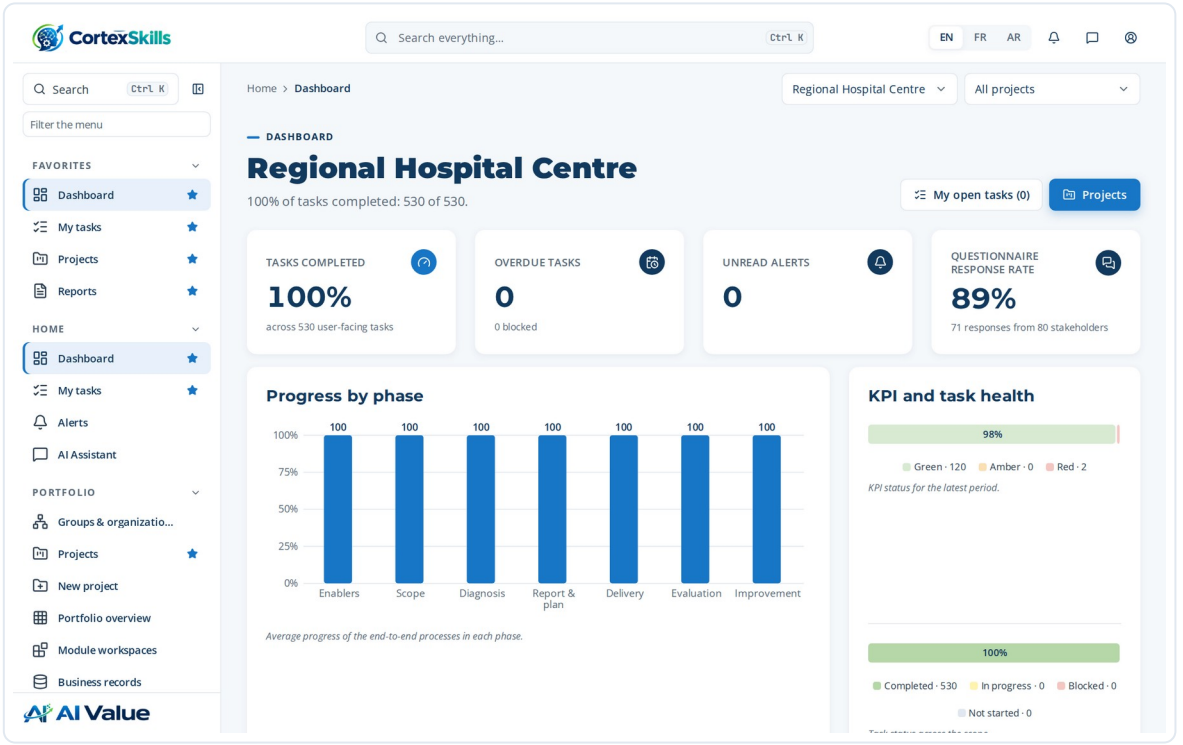


Training plan screen: programs, trainings and sessions.

The project workspace and the dashboard



Project workspace: phases, processes and progress.



Dashboard of the hospital.

CONCLUSION

What the hospital has at the end of the cycle

- A Training Engineering Report of 198 pages, built from 37 answers, 40 stakeholders and the diagnosis of every function.
- A locked Training Plan: 6 trainings, half-day agendas, a value proposition per persona and a budget of 1,260,000
- Evaluation at three Kirkpatrick levels, an ROI study, followed certifications and an internal audit with one minor nonconformity to correct.
- 37 published documents ready for an accreditation audit (ISO 7101, JCI), each traced to the data and the step that produced it.
- Next: waves 2 and 3 of the roadmap and the level-4 (results) evaluation six to twelve months after the trainings.

AI Value

Digital & AI Transformation

MEASURABLE • GOVERNED • SUSTAINABLE

Thank you

MEASURABLE · GOVERNED · SUSTAINABLE